Op. Dr. Murat Ayata
Kulak Burun Boğaz
Otolaryngology Head and Neck Surgery Specialist

Op. Dr. Murat Ayata Private Clinic
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INFORMED CONSENT FORM

Endolaryngeal Microsurgery

Microsurgery of the vocal cords / larynx (microlaryngoscopy)

Dear patient, this form has been prepared to explain, in **plain and clear language**, what you need to know about the procedure recommended to you. Its purpose is not to frighten you, but to help you make your decision **freely and on the basis of good information**.

Please read this form without rushing. Ask your doctor about anything you do not understand. Your doctor will also explain verbally any risks that are particular to your situation. Whether or not you give consent is entirely your right.

PATIENT AND PROCEDURE DETAILS

Full name: _____ Date of birth: _____

Passport / ID No: _____ File / Protocol No: _____

Telephone: _____ Date informed: _____

Planned procedure date: _____

Form first sent to patient (date / e-mail, WhatsApp, in person): _____

Doctor performing the procedure and providing information: **Op. Dr. Murat Ayata**

Hospital / facility where the procedure will be performed: _____

Lesion / preliminary diagnosis: _____

Side: ☐ Right vocal cord ☐ Left vocal cord ☐ Both sides
☐ Other area of the larynx _____

Planned technique: ☐ Cold instruments ☐ CO2 laser
☐ To be decided by the surgeon during the operation

Biopsy / pathological examination: ☐ Planned ☐ If considered necessary ☐ Not planned
_____

1 Your condition and the aim of the procedure

The larynx (voice box) sits at the front of the neck. It produces the voice, protects the airway and closes off the windpipe when you swallow. The vocal cords are two thin structures inside the larynx that vibrate to create sound.

Growths such as **nodules, polyps, cysts, Reinke's oedema, papillomas or leukoplakia (white patches)**, or a lesion that raises a suspicion of cancer, may develop on the vocal cords or elsewhere in the larynx. These can cause hoarseness, a tired voice, a feeling of something stuck in the throat and, rarely, breathing difficulty. Causes include incorrect or excessive voice use, smoking, reflux, infections (e.g. HPV) and sometimes unknown factors.

The aim of the operation is to remove the lesion while preserving as much healthy vocal cord tissue as possible, **to improve your voice**, to widen the airway where necessary and to reach a **definite diagnosis through pathological examination** of the removed tissue.

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2 How is the procedure performed?

The operation is carried out under general anaesthesia in an operating theatre, through the mouth; no cut is made in the neck.

1. **Reaching the larynx:** Once you are asleep, a metal tube (laryngoscope) is passed through your mouth to reach the larynx. Your teeth are protected with a special guard.
2. **Examination under the microscope:** The vocal cords are magnified with an operating microscope or endoscope and examined in detail. Additional findings not visible in the clinic may be found at this stage.
3. **Removing the lesion:** The lesion is removed with very fine microsurgical instruments or a CO2 laser, preserving the vibrating surface of the vocal cord as much as possible. If needed, only a small piece is taken for diagnosis (biopsy).
4. **Additional steps:** Depending on the situation, small medicated swabs may be used to stop bleeding, and medication may be injected into the vocal cord to reduce scarring.
5. **Finishing:** The laryngoscope is removed and the mouth and teeth are checked. There are no stitches or visible scars on the neck.

Very rarely, because of limited mouth opening, the shape of the teeth, or the anatomy of the jaw or neck, it may not be possible to reach the larynx safely. In that case the procedure may be stopped and alternative methods will be explained to you.

ANAESTHESIA	APPROXIMATE DURATION	HOSPITAL STAY / DISCHARGE
General anaesthesia	About 20–60 minutes	Usually same-day discharge or one night in hospital

3 Expected benefits and likelihood of success

- Less hoarseness and a clearer voice that tires less easily
- A **definite diagnosis** through pathological examination of the removed tissue, so that any disease found at an early stage can be treated in good time
- Easier breathing if the lesion is narrowing the airway

For benign lesions such as nodules, polyps and cysts, the voice improves significantly in most patients. However, the result depends on the type and size of the lesion, the previous condition of the vocal cords, factors such as smoking and reflux, and how well **voice rest and voice therapy** are followed after surgery. Some conditions, such as papillomas, can recur and may need more than one operation.

4 Other treatment options

- **Voice therapy (with a speech and language therapist):** May be the first choice, especially for vocal cord nodules, and often makes surgery unnecessary.
- **Medication:** Medicines that reduce stomach acid or inflammation may help where reflux or inflammation is the cause.
- **Stopping smoking and good voice care:** Helps some lesions (e.g. Reinke's oedema) to regress.
- **Office procedure under local anaesthetic:** For suitably selected small lesions or biopsies, a procedure can be done while you are awake using an endoscope through the nose or mouth; this is not suitable for every lesion.
- **Close observation:** Small lesions with no suspicion of cancer may be monitored with regular examinations.

5 What happens if the procedure is not performed?

If the operation is not performed, your hoarseness may continue or get worse. As the exact nature of the lesion is not known, diagnosis of an underlying serious condition (e.g. early-stage cancer or a lesion that could become cancerous) may, although rarely, be delayed. Growing lesions may rarely cause breathing difficulty.

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You have the right to decline the recommended procedure, to postpone it, or to seek a second medical opinion. This decision will not affect any other healthcare offered to you in any way. You may withdraw your consent at any time before the procedure begins; please be aware, however, that once a procedure has started there may be stages at which stopping would be medically unsafe.

6 Preparing for the procedure

- Blood tests will be carried out before the operation and you will be assessed by the anaesthetist.
- You must not eat or drink anything for **6–8 hours** before the operation (follow the anaesthetist's instructions).
- Be sure to tell us about any **loose teeth, crowns, bridges, implants or dentures**; dentures are removed before the operation.
- If reflux treatment has been recommended, you may be asked to start it before the operation.
- Tell your doctor about **all medicines** you take (including blood thinners, aspirin, painkillers, weight-loss and hormone medicines) and any **herbal products and supplements** (such as omega-3, vitamin E, ginkgo, garlic, ginseng, fish oil). Do not stop any medicine on your own without your doctor's advice.
- Be sure to tell us about any **allergies**, bleeding or clotting problems, heart, lung, diabetes, blood pressure or thyroid conditions, previous operations and any problems you have had with anaesthesia.
- If there is any chance you may be pregnant, or if you are breastfeeding, tell us before the procedure.
- Smoking and nicotine products significantly impair wound healing. If possible, stop at least 2–4 weeks before the procedure and do not smoke during recovery.

7 Recovery: what to expect

First 24–48 hours	You may have a sore or burning throat, mild discomfort when swallowing, hoarseness and a little blood-stained phlegm. You may notice numbness of the tongue, a change in taste or jaw pain; these usually settle within a few days.
Week 1	Your doctor will usually recommend **3–7 days of complete voice rest** (no talking, no whispering), followed by a gradual return to using your voice. Most people can return to desk work within 2–3 days; professional voice users (teachers, singers, call-centre staff) should wait longer.
Weeks 2–4	The voice gradually improves. Voice therapy is started if needed. Avoid shouting, singing and prolonged talking. The pathology result is usually available during this period and will be discussed with you.
Months 1–3	The vocal cords finish healing; the final voice usually becomes apparent within **6–12 weeks**. At the follow-up visit the vocal cords are examined with an endoscope.

8 Possible risks and complications

As with any medical procedure, this procedure carries some risks. Below, **expected temporary effects** and **complications** are listed separately and grouped by how often they occur. The figures given are **approximate** values based on the medical literature; your personal risk may be lower or higher depending on your age, general health, smoking and previous operations.

The great majority of complications are **temporary** and improve with appropriate treatment. Very rare but serious events are also included; being on this list does not mean they will happen to you, but you need to know about them to make an informed decision.

Expected, temporary effects (not complications — a normal part of healing)

- Sore throat and mild discomfort when swallowing (a few days)
- Temporary hoarseness or change in voice pitch after the operation
- A little blood-stained phlegm (first 1–2 days)

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- Tenderness of the jaw, lips or teeth; small cracks at the corners of the mouth

Common conditions (most are temporary or resolve with treatment)

- **Numbness of the tongue and change in taste** — Caused by pressure of the laryngoscope on the tongue; occurs in about **5–15 in 100** patients and usually settles within a few days to a few weeks, very rarely lasting several months.
- **Jaw joint pain** — Caused by keeping the mouth open for a long time; usually passes within a few days. It may be more frequent and longer-lasting in people with existing jaw joint problems.
- **Voice not improving as much as expected** — The voice does not always return fully to normal after surgery; voice therapy or further treatment may be needed.

Uncommon conditions

- **Dental damage** — Despite the use of a tooth guard, about **1–5 in 100** patients may have a chipped, cracked or loosened tooth, or damage to a crown/bridge. The risk is higher with loose, decayed or artificial teeth; dental treatment may be needed.
- **Worsening of the voice or permanent voice change** — If scar tissue forms on the surface of the vocal cord, the voice may become more hoarse, weaker or tire more quickly than before surgery. This may require voice therapy and sometimes further operations; rarely it may be permanent.
- **Recurrence of the lesion** — The lesion may come back, especially if voice misuse or smoking continues, or with conditions such as papillomas, and further surgery may be needed.
- **Web formation at the front of the vocal cords** — When both vocal cords are treated near the front, they may stick together; this can affect voice quality and a second operation may be needed to separate them.
- **Further treatment depending on the pathology result** — The pathology result may differ from what was expected (e.g. a lesion with a tendency to become cancerous, or cancer). In this case further surgery, radiotherapy or close follow-up may be needed; this will be explained to you in detail.

Rare or very rare, but potentially serious conditions

- **Swelling (oedema) of the larynx and breathing difficulty** — Swelling of the larynx may develop after surgery. It usually improves with medication; very rarely, observation in intensive care or a temporary opening in the neck to secure the airway (tracheostomy) may be needed.
- **Bleeding** — Significant bleeding is rare. Because of the risk of blood entering the airway, serious bleeding may require urgent intervention and a return to the operating theatre.
- **Laser-related risks** — When a CO2 laser is used, there is a risk of burns to surrounding tissue and, very rarely, of the breathing tube catching fire (airway fire). Laser-specific safety measures (special tube, wet swabs, eye protection) are applied. If this happened, it could cause serious burns to the airway and require prolonged intensive care treatment and a tracheostomy.
- **Injury to the mouth, tongue, lips or throat** — Minor injuries from the instruments usually heal on their own; very rarely stitches may be needed. Extremely rarely, a tear in the wall of the throat may let air escape under the skin of the neck (a crackling feeling) or cause a neck infection; this may require observation in hospital, intravenous antibiotics and, rarely, surgery.
- **Neck problems** — Because the head is tilted back, neck pain may occur — especially in people with cervical spine disease — and, very rarely, more serious spinal problems. Please tell us about any history of a slipped disc in the neck or neck surgery.

General risks that apply to any surgical / medical procedure

- **Anaesthesia risks:** If general anaesthesia or sedation is used, its risks (nausea, sore throat, dental damage, allergic reaction and, very rarely, heart or lung problems) will be explained separately by the anaesthetist, and your consent will be obtained on a **separate anaesthesia consent form**.

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- **Bleeding and blood transfusion:** The need for a blood transfusion in these procedures is very rare. In the event of serious bleeding, blood or blood products may be needed; these carry their own very rare risks (such as allergy or infection).
- **Infection:** A wound infection after surgery is usually treated with antibiotics; rarely, drainage (letting the pus out) may be needed.
- **Blood clots in the leg veins (DVT) and in the lungs (pulmonary embolism):** Very rare in short head and neck operations. Walking early reduces the risk. Hormone medicines, smoking, obesity and a previous history of clots increase the risk.
- **Drug allergy:** An allergic reaction may develop to the medicines, antiseptics, tapes or suture materials used.
- **Scarring and healing problems:** How scars form varies from person to person; very rarely a thick (hypertrophic) scar or keloid may form.
- **Risk to life:** Death in planned head and neck surgery is extremely rare today; however, no surgical procedure is entirely free of risk. A serious complication may require treatment in intensive care and may result in permanent disability.

This list does not include every possible complication; other unforeseeable and very rare events may occur. If a complication develops, your doctor will plan the necessary treatment and follow-up; this may require additional treatment, medication, procedures or a further operation.

9 When to contact us immediately

- **Shortness of breath**, or a whistling or harsh noise when breathing in (stridor)
- **Coughing up or vomiting blood**, or blood-stained phlegm increasing rather than decreasing
- **Inability to swallow**, or pain or swelling so severe that you cannot swallow your saliva
- **Fever above 38.5 °C** with a sore throat that keeps getting worse
- Swelling of the neck or face, or a crackling feeling under the skin

If any of these occur, call your doctor without delay: +90 288 502 50 20 · WhatsApp +90 555 333 21 20. If you cannot reach us, go to the nearest emergency department or call 112 (the emergency number in Türkiye and across Europe).

10 Medication and everyday-life advice

- Follow the recommended **voice rest** strictly; whispering also strains the vocal cords. You can communicate in writing or with phone apps.
- Drink plenty of water and keep the air in your room humid. Avoid throat clearing and coughing as much as possible.
- Stay away from smoking and cigarette smoke; smoking both impairs healing and increases the risk of the lesion returning.
- If reflux medication has been prescribed, take it for the recommended period; do not eat within 2–3 hours before bedtime and limit spicy and acidic foods.
- Air travel is usually safe from the following day; continue voice rest during the journey.
- Take the medicines prescribed for you (antibiotics, painkillers, etc.) at the recommended dose and for the recommended time. Do not take aspirin or similar medicines that may increase bleeding without asking your doctor.
- On flights or journeys longer than 4 hours after surgery, drink plenty of water, move around regularly and wear compression stockings if advised; this reduces the risk of leg clots.
- During recovery you are advised not to drink alcohol and to avoid smoking and nicotine products.

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- Please attend all follow-up appointments. If you have travelled from abroad, follow-up after your return can continue by online consultation and photos/videos; if you need to see a local doctor, you will be given a written operation report and our recommendations.

11 Unexpected findings during the procedure, tissue samples and blood products

If an unforeseen situation arises during the procedure, your doctor will stay within the scope of the procedure you have consented to on this form. **Unless there is a risk to life or of serious harm to an organ**, no additional procedure that you have not consented to will be performed; in such a case the procedure will be brought to a close at a safe point, and you will be informed afterwards so that the decision can be made together with you. If your life is in danger, the emergency treatment needed to protect your life or the organ will be carried out.

Tissues removed during the procedure will be sent for **pathological examination** where necessary to confirm the diagnosis. Additional treatment or follow-up may be needed depending on the result.

Blood transfusion is very rarely needed in these procedures and will only be given when it is vital. If you do not accept blood or blood products (for example for religious reasons), you must tell us before the procedure; the possible consequences of this decision will be discussed with you separately and recorded under “Information specific to you”.

12 Photographs, image records and your personal data

Photographs/videos may be taken before and after the procedure for planning, follow-up and assessment of the result. These records form part of your medical file and are kept confidential. Your images will **never be used for advertising, promotion or social media.**

Your images may be used for any other purpose **only with your separate and explicit permission**. Declining will not affect your treatment in any way, and you may withdraw your permission at any time:

☐ I agree that my images are used **only for medical records and treatment follow-up**.

☐ I permit my images to be used for scientific publication and medical education **in a way that does not identify me** (face/eye area covered).

☐ I permit my images to be used for scientific publication and medical education **in a way that may identify me**.

☐ I **do not permit** scientific/educational use.

Your personal and health data are processed under the Turkish Personal Data Protection Law No. 6698 (KVKK) and the relevant health legislation, for the purposes of diagnosis, treatment and legal obligations, by persons bound by professional confidentiality. You may consult our Data Protection Information Notice for details.

13 Information and notes specific to you (completed by the doctor)

Patient-specific additional risks (co-existing diseases, medicines, previous operations), details of the plan, the patient's expectations, questions asked and answers given. If there is nothing to add, write “none”.

14 Patient declaration

☐ I have read and understood this form in my own language. I was given enough time to think; I had the opportunity to ask questions and received satisfactory answers.

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- ☐ My condition, the aim of the recommended procedure, how it will be done, by whom and where, its approximate duration, expected benefits, likelihood of success, alternatives and what may happen if it is not done have been explained to me verbally and in writing.
- ☐ The common and potentially serious risks, and the possible consequences of these complications (including that further treatment or a repeat procedure/operation may be needed), have been explained to me. I understand that the outcome of any medical procedure **cannot be 100% guaranteed**.
- ☐ I have given accurate and complete information about my health, the medicines I take, my allergies and past illnesses. I understand that incomplete or incorrect information may increase the risks.
- ☐ I understand that following the instructions given before and after the procedure plays a part in the result, and that attending follow-up appointments is important.
- ☐ I know that I can withdraw my consent at any time before the procedure begins. I understand that signing this form does not remove the doctor's duty of care.
- ☐ I am making this decision of my own free will, without pressure from anyone.
- ☐ I understand that if it becomes clear during the operation that the larynx cannot be reached safely, the procedure may be stopped, and that further treatment or follow-up may be needed depending on the pathology result of the removed tissue.
- ☐ I understand that the final voice also depends on my following voice rest, voice therapy, stopping smoking and reflux treatment after surgery.

- ☐ **I GIVE MY CONSENT to the procedure described above.**
- ☐ **I DO NOT ACCEPT the procedure described above. The possible consequences of refusing have been explained to me.**

We recommend the patient writes in their own handwriting: "I have read and understood this form and I give my consent."

15 Signatures

	FULL NAME	SIGNATURE	DATE / TIME
Patient			
Legal representative (parent / guardian) — relationship:			
Doctor providing information and performing the procedure			
Interpreter (if needed) — language:			
Witness (if needed)			

Interpreter's statement: I have translated this form and the doctor's verbal explanations to the patient / legal representative completely and accurately, in a language they understand.

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FOR PATIENTS UNDER 18 OR WITH LIMITED DECISION-MAKING CAPACITY

For patients under 18, consent is given by a **parent or legal guardian**. The child is informed in a way appropriate to their age and maturity and their view is sought. For patients lacking capacity, consent is given by the legal representative.

☐ If we share parental responsibility, I declare that the other parent is aware of and agrees to this procedure.

☐ I declare that there is no court order or dispute concerning custody / guardianship (if there is, the relevant document has been shown to the doctor).

The child was informed in an age-appropriate way; the child's view: _____

This form has been prepared in two copies: one is kept in the patient's file and one is given to the patient / legal representative.

WITHDRAWAL OF CONSENT (COMPLETE ONLY IF YOU CHANGE YOUR MIND)

I withdraw the consent I gave above. The possible consequences of withdrawing have been explained to me.

Name, signature, date and time of patient/representative withdrawing consent: _____

This form has been prepared in accordance with the Turkish Patient Rights Regulation and the informed consent principles of the Turkish Medical Association. It complements, but does not replace, the verbal explanation given by your doctor.